



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : غاده غنيم محمود سالم
المحافظة : القاهرة
الحى / المدينة : دار السلام
رقم البطاقة : 27709010108803 رقم التليفون : 01148864478
العنوان : ...
العمر (تاريخ الميلاد) : 01-09-1977 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 30-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج الالم ومضادات القئ لمرضي
الاورام المنتشر [14456]

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / د. / د. / د. / د. / د. / د. / د. / د. / د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: غاده غنيم محمود سالم National ID: 27709010108803	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Pin Medication And Anti Emetics
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





MDT FORM

22/12/2025 G

Name	غادة عفيف محمد	MRN	٢٠٩/٢٠١٨
Age	47y.	ID	٢٧٧٠٩٠١٠١٠٨٠٢
Menopausal status	premenop	Phone	0114 8864478
NRS SCORE		Marital status	011275 42609
BMI		Family history	
CO morbidity		REASON OF MDT	

Clinical Data

BX: IDC				R PSI: 4		L	
ER: +x 6/8	PR: +x 6/8	HER 2: 3+	Ki67: 30%	→ HFL Biopsy on 3/2025			
Sonomammography: premenop Met. Breast cancer to Liver Lung with Dp of Bilobar HFL							
Mets Workup: History 2018 :- Left Breast cancer cTN, TNM cT1N1M0 ER+, PR+, HER2+ Rec 4 FAC → LR BUS 6/2018 → ady Taxotere x 2, Herceptin x 12 Cx 12/2019 TAM f 9/2019 → 1/2022 1/2022 :- Mets to Lung & Liver 18 Wks Taxol + Herceptin → RD.							
Recommendations: AT + Zoladex + Herceptin till 3/2023 Newly developed HFL Taxol + Herceptin → 5/2023 Dp → Xeloda + Tykerb X10 Cx → oral Navelbe X3 → Dp. → 3 cycles Gemzar → SD. Kadcyb f 4/2024 → 2/2025 x12 Cx → Dp. Carboplat x 6 Cx + 4 phergo. → Dp Rec 7 Wks Taxol + 2 phergo → Dp of Bilobar HFL for HDT LKTN0							
MDT OUTCOME & PLAN New Pretreated for the chemo with Gemzar, if for tech the with Gemzar + Herceptin for 12 Wks / Consider TDX-D for NBO.							

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric



Patient Name	: الأستاذة / غادة خنيم محمود سالم	Request No.	: 13008160
Patient No.	: 27709010108803	Request Date	: 08/03/2025
Path No.	: 948/25	Verification Date	: 19/03/2025
Sex	: Female	Financial Category	: أورام دار السلام - نفقة الدولة
Age	: 47 Y	Department	: Out Patient
Referring Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Known case of breast cancer with hepatic focal lesion.

Nature of Specimen: Tru-cut biopsy, collected at 4/3/2025.

Gross Examination:

- Multiple grayish white tissue cores up to 1 cm, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:

- * Revealed cores of liver tissue showed positive metastatic deposits.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * IPT (Positive control showed appropriate reactivity):
- GATA3: Positive.
- ER: Moderate nuclear reaction in 60% of tumor cells (score 6 /8).
- PR: Strong nuclear reaction in 11% of tumor cells (score 6 /8).
- Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).
- Ki67: 30% nuclear reaction of tumor cells.

Diagnosis:

Liver tissue, Tru-cut biopsy:

Positive for metastatic deposits.

IPT:

- GATA3: Positive.
- ER: Positive (score 6 /8).
- PR: Positive (score 6 /8).
- Her2: Positive (score 3+).

Verified by :

DR.Menna Nabil

Printing Date : 23/03/2025 09:45:24

Page 1 of 2

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خريطة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report

Prof. Aml Hareedy
Dr. Mennatullah Atef

Printing By : Ali mostafa

Patient Name : الأستاذة / غادة خنوم محمود سالم
Patient No. : 27709010108803
Path No. : 948/25
Sex : Female
Age : 47 Y
Referring Doc :

Request No. : 13008160
Request Date : 08/03/2025
Verification Date : 19/03/2025
Financial Category : نفقة الدولة - اورام دار السلام
Department : Out Patient
Registered by : Mrs. Mayada Hegazy

Pathology Report

- Ki67: Positive in 30% of tumor cells.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Prof. Dr. Lubna Omar.
Dr. Nermeen Mohammed.

Verified by:
DR. Menna Nabil

Printing Date : 23/03/2025 09:45:24

Page 2 of 2

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خطابات أورام
Patient Name : غاده غنيم محمود

sex : female Age: 50

Hosp.Word : TC

Patient status : outpatient

Path.No. : 0000

IHC . No : 10434

Referring Doctor: اورام السلام

Date : 23-05-2018

Provisional Diagnosis: Request for ER/PR/cerbB2

Immunohistochemistry Report

IMMUNOHISTOCHEMISTRY

Immunostaining effectuated on 5 µm thick paraffin sections, the heat induced epitope retrieval using microwave, the ChemMate DAKO Envision, DAKO autostainer and the following antibodies:

- Estrogen receptors: moderate nuclear staining of 50% of neoplastic cells.
- Progesterone receptors: moderate nuclear staining of 10% of neoplastic cells.
- CerbB2: strong complete membranous staining of all the neoplastic cells.

CONCLUSION

Invasive mammary carcinoma, core biopsy:

- Estrogen and Progesterone receptors positive.
- CerbB2 positive.

NB

core biopsy, score is not applicable

Enclosed :

Slides :

Blocs :

مدير معمل الصبغات الهيمتومناعية

د. / هويدا عبدالعال

Director Immunohistochemical Lab

Prof. Howayda Abd El Ali

30-05-2018

Name: ghada ghameem mahmoud

Date: 11 June 2020

Soft tissue ultrasound examination of both breasts revealed: -

Follow up after left breast lumpectomy revealed:

Left breast:

- > Skin thickening seen at the upper outer quadrant and areolar region
- > The breast parenchymal shows multiple hypoechoic line of interstitial edema
- > There is irregular hypoechoic area under the surgical scar measuring 11x12x16 mm, with no calcification (? Surgical scar).
- > No other mass or cystic changes.
- > One enlarged axillary LN seen, measuring 21x9 mm, it has fatty hilum and thin cortex..

Right breast:

- > Predominantly fibro-glandular breast parenchyma with no areas of architectural distortion.
- > No masses or cysts could be detected.
- > Normal duct system.
- > Normal nipple areola complexes.
- > Intact skin and subcutaneous tissues.
- > No enlarged right axillary LNs..

OPINION:

Edematous changes seen at the left breast with ? surgical scar for close follow up BIRADS III.

Best regards

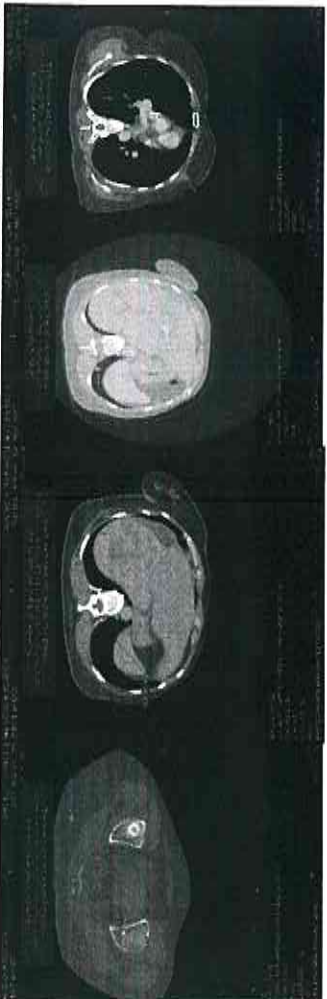
Dr / Deaa Ahmed MS-FEBR

The Breast Imaging and Reporting Data System for classifying the radiological diagnosis of breast lesions:

- BIRADS 0: needs further investigations (0%).
- BIRADS 1: normal findings.
- BIRADS 2: definitely benign.
- BIRADS 3: probably benign (2 %).
- BIRADS 4: probably malignant (50 %).
- BIRADS 5: malignant (98-100 %).
- BIRADS 6: proven pathologically malignant.

Patient's Name:
Date:
Referred by:
Hospital:

Ghada Ghoneim Mahmoud Salem
2026-08-03
Prof.Dr. no doctor
Goustave Roussey International Egypt



MULTISLICE CT EXAMINATION OF THE CHEST ,ABDOMEN AND PELVIS WITHOUT CONTRAST ADMINISTRATION REVEALED:-

- **The right thyroid lobe lesion could not be delineated well in the current study.**
- Evident left breast surgical intervention with scarring . No evidences of either residual/newly-developed/recurrent neoplastic lesions seen herein. The skin is still mildly thickened and measures about 3.4 mm.
- Normal right breast.
- **Benign-looking bilateral axillary lymph nodes are seen with preserved their hilar fat contents and the largest one measures about 14 mm on the right side.**
- Hyperinflated both lungs.
- Mild reticulations are seen in the anterior segment of left upper pulmonary lobe.
- Thin linear atelectatic bands in the middle lobe , in the lingula and in the posterior segment of right lower pulmonary lobe.
- Newly-developed tiny pulmonary nodules are seen in the anterior segment of left upper and the subpleural posterior segment of left lower pulmonary lobe. The later is larger and measures about 9mm.
- No other sizable pulmonary nodules ,patchy area of consolidation or foci or calcifications noted.
- No sizable abnormally-enhancing mediastinal or hilar lymph nodes seen.
- Patent central airways.
- No pleural or pericardial effusions.
- **Average size of the heart with normal CT of its big vessels.**
- A newly-developed tiny lesion of osteosclerosis is seen in the posterior limb of left acetabulum **and the scanned spine shows evidences of spondylo-degenerative changes.**
- Average sized right lobe and enlarged left lobe of the beaver-tail liver, measuring about 15 cm and 15.8cm respectively , showing a non-homogeneous attenuation and enhancement pattern.
- A newly-developed subcapsular collection is seen ,overlying the dome of the liver and measures about 73x21mm in dimensions.
- Reduced the size of the right and to lesser extent left hepatic lobe neoplastic lesion. Today it measures about 12.6x8.6 cm in dimensions.(versus 18x15cm in the previous study).
- No other newly-developed sizable hepatic focal lesions or dilated biliary radicles.
- Patent PV.
- Average sized spleen,measuring about 6.4 cm. No focal lesions seen inside.
- Thickened the left adrenal gland.
- Normal CT appearance of the pancreas, functioning both kidneys, right adrenal gland, aorta and IVC.
- No sizable abdominal or pelvic lymphadenopathy.
- No ascites.
- No intra-vesical stones,solid masses,wall thickening or diverticulosis seen.
- The rest of the pelvic structures are unremarkable.

Patient's Name:

Ghada Ghoneim Mahmoud Salem

Date:

2026-08-03

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

IMPRESSIONS:

A status follow up ,and compared with the lastly done study at 06/04/2026,the current examination shows:

- Evident left breast surgical intervention with scarring . No evidences of either residual/newly-developed/ recurrent neoplastic lesions seen herein. The skin is still mildly thickened .
- Hyperinflated both lungs with mild reticulations in the anterior segment of left upper pulmonary lobe,likely to be a post therapeutic sequelae.
- Thin linear atelectatic bands in the middle lobe , in the lingula and in the posterior segment of right lower pulmonary lobe.
- Newly-developed tiny pulmonary nodules in the anterior segment of left upper and the subpleural posterior segment of left lower pulmonary lobe, thickened the left adrenal gland ,and a newly-developed tiny lesion of osteosclerosis in the posterior limb of left acetabulum,likely to be metastatic in nature.
- A newly-developed subcapsular collection overlying the dome of the liver and reduced the size of the right and to lesser extent left hepatic lobe neoplastic lesion,denoting a partially regressive course.

Prof/Hanan El-Ahwal.

ECHO REPORT

NAME: غادة عيسى محمود DATE: 25 / 3 / 2026
 ID NUMBER: 309 / 18 SEX: ♀ AGE: 48

F	64%	55-75%	PWT	0.8	0.6-1.1cm
VEDD	4.6	3.5-5.8cm	AO.R	2.3	3-3.7cm
VESD	2.3	2-4cm	LA	3.6	2-4cm
S	30%	25-45%	RV		0.7-2.3cm(supine)
WT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions Normal
 LV Wall Thickness Normal
 LV Diastolic Function Normal
 SWMA No RWMA
 LA and Ao Root Dimensions Normal
 Mitral Valve N
 Aortic Valve N
 Tricuspid Valve N
 Pulmonary Valve N
 Right Side and Systolic Function TAPE: 2.6
 Intracardiac Masses and Effusion

DR. غادة عيسى محمود

Patient's Name: ghada ghonaim mahmoud salem
Date: 2026-04-06
Referred by: Prof.Dr. ct cap iv
Hospital: Goustave Roussey International Egypt

MULTISLICE POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

TECHNIQUE: Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

FINDINGS:

Clinically reported leftbreast carcinoma , compared to last study dated on 23/11/2025

(I) Chest:

- Still noted left breast scar tissue 1.7 x 1.7 cm inseparable from pectoralis muscle .
- Left upper lobe and lingula subpleural reticulation likely post radiotherapy .
- Clear both lung fields .no sizable pulmonary masses or consolidation patches
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.
- Right thyroid nodule measures 18 x 15 mm .

(II) Abdomen and pelvis:

- The liver is enlarged in size showing homogenous CT density , progressive course of large segment VIII heterogeneously enhancing mass involving segment Iv & V also measures 8.7 x 5 x 5 cm compared to 7 x 4.8 cm at last study .

Patient's Name: ghada ghonaim mahmoud salem
Date: 2026-04-06
Referred by: Prof.Dr. ct cap iv
Hospital: Goustave Roussey International Egypt

- Patent and homogenously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.
- Normal CT appearance of the pancreas, both kidneys, aorta and IVC.
- Normal CT appearance of the spleen .
- Regular filling of the urinary bladder showing no stones, masses .
- Unremarkable other pelvic organs.
- No retro-crural, para-aortic or pelvic lymphadenopathy.
- No ascites.
- Small sclerotic lesion at the left iliac bone likely bone island
- No vertebral structural collapse .

OPINION:

- Progressive course regarding size of hepatic lobe focal lesion rather stationary rest of the study .

Dr.Reham Khedr, MD

Patient Name	غادة غنيم محمود	Age	48
Sex	F	Special habits	
Family History		Comorbidity	
Surgical History		Telephone	01127542609
Menopausal Status	Pre-menopausal	Marital Status	
ID		Medical ID	308/2018
Date	28/9/2025	Reason for MDT	REMDT

Medical Report

Pathology: IDC

ER : +ve 8/8
PR : +ve 8/8
Her2 : 3+
Ki67 :

PREMENOPAUSAL MRC TO LUNG & LIVER HISTORY – 2018 : CT3N1M0
Neo.adj. 4FAC -> LT BCS 6/2018 -> adj 2 taxotere stopped because of AE -> PORT +17
cycles herceptin last 12/2019 started tamoxifen 9/2018 to 1/2022
1/2022 dyspnea CT -> lung nodules + HFL
Received 18 weeks taxol + herceptin -> regressive course
Started AI + ZOLADEX + HERCEPTIN from 22/6 till 22/3/2023 CTS -> PD
3/2023 -> METS HFL -> received 9 weeks taxol + herceptin up to 22 cycle
of HFL
Started taxol + carbo + herceptin x3 cycles -> 5/2023 PD
xeloda + tykerb x3 cycles

-> RD
Now in C9 xeloda,6tykerb 29/11/2023 ct:pf of HFL
Nave;bine x3 cycles -> PD. PS 1- normal LFT NO .VIS. CRISIS-HFL =6cm
3cycle gamzar -> SD -> received kadcyla with very good response recieved12 cycles
CARNOPLATIN SINGLE AGENT ER PR 618 HER2 +3
US GUIDED BIOPSY FROM HFL
NEW CT PD IN LIVER REC 1 CYCLE CARBOPLATIN

- For enhertu at NGO / start paclitaxel weekly meanwhile

MDT
DECISION

Name	د/ خالد عبدالعزيز	د/ احمد نجيب	د/ رانيا شفيق
Signature			

Patient Name: Ghada Ghonaim Mahmoud Salem	Date : 22/09/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 43 Year(s)
Medical ID: 309/2018	Pat. ID: 13109

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF

THE CHEST, ABDOMEN AND PELVIS REVEALED:

Status post operative of left BCS showed
Compared to the last CT examination dated 15/5/2025 (hard copy
with no DICOM series), the current study revealed

I- CHEST:

- Stationary course of the previously noted left operative bed scar tissue measures about 1.7x1.4 cm in its maximum TV,AP dimensions respectively.
- seen adherent/inseparable from pectoralis muscle with mild skin retraction.
- Stationary course of the previously noted left apical fine reticulation and bilateral basal lung lobes atelectatic bands.
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- The liver is average in size showing homogenous parenchymal attenuation.
 - Stationary course regarding size and density of the previously noted right hepatic segment VIII hypodense discrete and confluent focal lesion, the largest measures about 3x2.5 cm.
 - Newly developed left hepatic lobe segment IVb hypodense confluent lesions measures about 2.3x3.4 cm
- No dilated biliary radicles.
- Both kidneys are normal size, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.

Patient Name: Ghada Ghonaim Mahmoud Salem	Date : 22/09/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 43 Year(s)
Medical ID: 309/2018	Pat. ID: 13109

- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- Pelvic calcific phleboli
- No ascites.

Opinion:

- Newly developed left hepatic focal lesion as described for further dynamic MRI and comparison with unavailable previous DICOM.
- Stationary course of the rest of examination.

Dr.Norhan Ramzy,MD



Presidential Initiative for Women's Health - Pathology Lab
المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي



Patient Name	الأستاذة / غاده غنيم محمود سالم	Request No.	: 138127
Patient No.	: 27709010108803	Request Date	: 11/01/2022
Path No.	: 103/2022	Verification Date	: 26/01/2022
Sex	: Female	Financial Category	: نفقة الدولة - اورام دار السلام
Age	: 44 Y	Department	: Out Patient
Referreing Doc :		Registered by	: marwa mohamed youss

Pathology Report

- Histologic response to therapy: N/A
- Additional pathologic findings: N/A

Diagnosis:

Left breast mass, Tru-cut biopsy:

Totally fibrosed cores (scar tissue), free of malignancy.

Prof. Aml Hareedy
Dr. Mennatullah Atef

Verified by :

Printing Date : 26/01/2022 13:29:39 Page 2 of 2 Printing By : Ali mostafa



Patient Name	الأستاذة / غاده غنيم محمود سالم	Request No.	: 13008160
Patient No.	: 27709010108803	Request Date	: 08/03/2025
Path No.	: 948/25	Verification Date	: 19/03/2025
Sex	: Female	Financial Category	: نفقة الدولة - اورام دار السلام
Age	: 47 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Known case of breast cancer with hepatic focal lesion.

Nature of Specimen: Tru-cut biopsy, collected at 4/3/2025.

Gross Examination:

- Multiple grayish white tissue cores up to 1 cm, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:

- * Revealed cores of liver tissue showed positive metastatic deposits.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * IPT (Positive control showed appropriate reactivity):
 - GATA3: Positive.
 - ER: Moderate nuclear reaction in 60% of tumor cells (score 6 /8).
 - PR: Strong nuclear reaction in 11% of tumor cells (score 6 /8).
 - Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).
 - Ki67: 30% nuclear reaction of tumor cells.

Diagnosis:

Liver tissue, Tru-cut biopsy:
Positive for metastatic deposits.

IPT:

- GATA3: Positive.
- ER: Positive (score 6 /8).
- PR: Positive (score 6 /8).
- Her2: Positive (score 3+).

Verified by :
DR.Menna Nabil

Printing Date : 23/03/2025 09:45:24

Page 1 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name	الأستاذة / غاده غنيم محمود سالم	Request No.	: 13008160
Patient No.	: 27709010108803	Request Date	: 08/03/2025
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Sex	: Female	Financial Category	: نفقة الدولة - اورام دار السلام
Age	: 47 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

- Ki67: Positive in 30% of tumor cells.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Prof. Dr. Lubna Omar.

Dr. Nermeen Mohammed.

Verified by :

DR.Menna Nabil

Printing Date : 23/03/2025 09:45:24

Page 2 of 2

Printing By : Mrs.Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name: Ghada Ghonaim Mahmoud Salem	Date : 23/11/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 43 Year(s)
Medical ID: 309/2018	Pat. ID: 13109

MULTISLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

Status post operative of left BCS showed Compared to the last CT examination dated 22/9/2025, the current study revealed

I- CHEST:

- Stationary course of the previously noted left operative bed scar tissue measures about 1.7x1.4 cm in its maximum TV,AP dimensions respectively, seen adherent/inseparable from pectoralis muscle with mild skin retraction.
- Stationary course of the previously noted left apical fine reticulation and bilateral basal lung lobes atelectatic bands.
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- The liver is average in size showing homogenous parenchymal attenuation.
- Progressive course regarding size and extension of the previously noted right hepatic segment VIII ill-defined heterogenous confluent focal lesion seen extending to the left lobe segment IV , V, VII measuring collectively 7 x 4.8 cm compared to 6.2 x 3.6 cm .
- No dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- Pelvic calcific phleboli
- No ascites.



- No worrisome lytic or sclerotic bony lesions .
- No vertebral structural collapse or wedging .

Opinion:

- Progressive course regarding size and extension of the bilobar hepatic infiltrative mass
- Stationary course of the rest of examination.

Dr.Reham Khedr ,MD

Patient Name: Ghada Ghonaim Mahmoud Salem	Date : 22/09/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 43 Year(s)
Medical ID: 309/2018	Pat. ID: 13109

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF

THE CHEST, ABDOMEN AND PELVIS REVEALED:

Status post operative of left BCS showed

Compared to the last CT examination dated 15/5/2025, the current study revealed

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- seen adherent/inseparable from pectoralis muscle with mild skin retraction.
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- No pulmonary nodules, masses or cavitary lesions.
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- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- The liver is average in size showing homogenous parenchymal attenuation.
- Progressive course regarding size and extension of the previously noted right hepatic segment VIII ill-defined heterogeneous confluent focal lesion seen extending to the left lobe segment IV with largest component measuring collectively 6.2 x 3.6 cm compared to about 3x2.5 cm.
- No dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.

Patient Name: Ghada Ghonaim Mahmoud Salem	Date : 22/09/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 43 Year(s)
Medical ID: 309/2018	Pat. ID: 13109

- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- Pelvic calcific phleboli
- No ascites.

Opinion:

- Progressive course regarding size and extension of the previous ill-defined right hepatic lobe focal lesion with extension to left lobe in current study.
- Stationary course of the rest of examination.

Dr. Norhan Ramzy, MD

Signature Prof. Ali Remah

13/11/2025 06:59

Patient Name	الاستاذ / غاده غنيم محمود سالم	Request No.	: 138127
Patient No.	: 27709010108803	Request Date	: 11/01/2022
Path No.	: 103/2022	Verification Date	: 26/01/2022
Sex	: Female	Financial Category	: نفقة الدولة
Age	: 44 Y	Department	: Out Patient
Referreing Doc :		Registered by	: marwa mohamed youss

Pathology Report

◦ Histologic response to therapy: N/A

◦ Additional pathologic findings: N/A

Diagnosis:

Left breast mass, Tru-cut biopsy:

Totally fibrosed cores (scar tissue), free of malignancy.

Prof. Aml Hareedy
Dr. Mennatullah Atef

Verified by :

Printing Date : 26/01/2022 13:29:39

Page 2 of 2

Printing By : Ali mostafa

خطابات أورام

Patient Name : غاده غنيم محمود

sex : female Age: 50

Hosp. Word : TC

Patient status : outpatient

Path.No.: 0000

IHC. No : 10434

Referring Doctor: اورام السلام

Date : 23-05-2018

Provisional Diagnosis: Request for ER/PR/cerbB2

Immunohistochemistry Report

IMMUNOHISTOCHEMISTRY

Immunostaining effectuated on 5 µm thick paraffin sections, the heat induced epitope retrieval using microwave, the ChemMate DAKO Envision, DAKO autostainer and the following antibodies:

- Estrogen receptors: moderate nuclear staining of 50% of neoplastic cells.
- Progesterone receptors: moderate nuclear staining of 10% of neoplastic cells.
- CerbB2: strong complete membranous staining of all the neoplastic cells.

CONCLUSION

Invasive mammary carcinoma, core biopsy:

- Estrogen and Progesterone receptors positive.
- CerbB2 positive.

NB

core biopsy, score is not applicable

Enclosed :

Slides : Blocs :

مدير معمل الصبغات الهستو مناعية

أ. د / هويدا عبدالعال

Director Immunohistochemical Lab

Prof. Howayda Abd El All

30-05-2018

Name: ghada ghomeem mahmoud
Date: 11 June 2020

Soft tissue ultrasound examination of both breasts revealed: -

Follow up after left breast lumpectomy revealed:

Left breast:

- Skin thickening seen at the upper outer quadrant and areolar region
- The breast parenchymal shows multiple hypoechoic line of interstitial edema
- There is irregular hypoechoic area under the surgical scar measuring 11x12x16 mm, with no calcification (? Surgical scar).
- No other mass or cystic changes.
- One enlarged axillary LN seen, measuring 21x9 mm, it has fatty hilum and thin cortex..

Right breast:

- Predominantly fibro-glandular breast parenchyma with no areas of architectural distortion.
- No masses or cysts could be detected.
- Normal duct system.
- Normal nipple areola complexes.
- Intact skin and subcutaneous tissues.
- No enlarged right axillary LNs..

OPINION:

Edematous changes seen at the left breast with ? surgical scar for close follow up BIRADS III.

Best regards

Dr / Daaa Ahmed MS-FEBR

The Breast Imaging and Repoting Data System for classifying the radiological diagnosis of breast lesions:

BIRADS 0: needs further investigations (0%).

BIRADS 1: normal findings.

BIRADS 2: definitely benign.

BIRADS 3: probably benign (2 %).

BIRADS 4: probably malignant (50 %).

BIRADS 5: malignant (98-100 %).

BIRADS 6: proven pathologically malignant.



Patient Name : الأستاذة / غاده غنيم محمود سالم :

Patient No. : 27709010108803

Path No. : 948/25

Sex : Female

Age : 47 Y

Referreing Doc :

Request No. : 13008160

Request Date : 08/03/2025

Verification Date : 19/03/2025

Financial Category : نفقة الدولة - اورام دار السلام

Department : Out Patient

Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Known case of breast cancer with hepatic focal lesion.

Nature of Specimen: Tru-cut biopsy, collected at 4/3/2025.

Gross Examination:

- Multiple grayish white tissue cores up to 1 cm, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:

- * Revealed cores of liver tissue showed positive metastatic deposits.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * IPT (Positive control showed appropriate reactivity):
 - GATA3: Positive.
 - ER: Moderate nuclear reaction in 60% of tumor cells (score 6 /8).
 - PR: Strong nuclear reaction in 11% of tumor cells (score 6 /8).
 - Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).
 - Ki67: 30% nuclear reaction of tumor cells.

Diagnosis:

Liver tissue, Tru-cut biopsy:

Positive for metasatic deposits.

IPT:

- GATA3: Positive.
- ER: Positive (score 6 /8).
- PR: Positive (score 6 /8).
- Her2: Positive (score 3+).

Verified by :

DR.Menna Nabil

Printing Date : 23/03/2025 09:45:24

Page 1 of 2

Printing By : Mrs.Mayada Hegazy



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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name	الأستاذة / غاده غنيم محمود سالم	Request No.	: 13008160
Patient No.	: 27709010108803	Request Date	: 08/03/2025
Path No.	: 948/25	Verification Date	: 19/03/2025
Sex	: Female	Financial Category	: نفقة الدولة - اورام دابر السلام
Age	: 47 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

- Ki67: Positive in 30% of tumor cells.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

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Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Prof. Dr. Lubna Omar.

Dr. Nermeen Mohammed.

Verified by :

DR.Menna Nabil

Printing Date : 23/03/2025 09:45:24

Page 2 of 2

Printing By : Mrs.Mayada Hegazy



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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name: Ghada Ghonaim Mahmoud Salem	Date : 22/09/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 43 Year(s)
Medical ID: 309/2018	Pat. ID: 13109

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF

THE CHEST, ABDOMEN AND PELVIS REVEALED:

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Compared to the last CT examination dated 15/5/2025, the current study revealed

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Patient Name: Ghada Ghonaim Mahmoud Salem	Date : 22/09/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 43 Year(s)
Medical ID: 309/2018	Pat. ID: 13109

- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- Pelvic calcific phleboli
- No ascites.

Opinion:

- Progressive course regarding size and extension of the previous ill-defined right hepatic lobe focal lesion with extension to left lobe in current study.
- Stationary course of the rest of examination.

Dr. Norhan Ramzy, MD

Signature Prof. Ali Remah

13/11/2025 06:59

Patient Name: Ghada Ghonaim Mahmoud Salem	Date : 20/05/2023
Exam : C.T CHEST WITH CONTRAST	Age : 41 Year(s)
Medical ID:	13109

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED:-

**** A known case of breast cancer.**

**** As compared to the previous study dated 06/03/2023,**

1. Chest:

Status post left BCS intervention,

- **Stable appearance of the operative bed** ill-defined scar close to the pectoralis muscle.
- **Still noted** right upper lobe posterior segment reticulonodular density.
- **Still noted** left lung upper lobe anterior subpleural fine reticular bands with right middle atelectatic band. Right lower lung subpleural mosaic pattern.
- **Still noted** right lower lobe atelectasis.
- No hilar or mediastinal sizable lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size. No pericardial effusion detected.
- Normal non-enhanced appearance

2. Abdomen and pelvis:

- The liver is enlarged with **progressive course** regarding the size of the two hypo-enhancing focal lesions, the largest is currently appreciated subcapsular at segment V subcapsular measuring about 33x28 mm as compared to ill defined 18 mm, and segment VI currently measuring about 21x11 mm compared to 15x11 mm. No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.

جمهورية جزر القمر
بطاقة تحقيق الشخصية

غادة
غنيمة محمود سالم

٦ ش سيد سليمان - جزيرة بين البحرين
الجزيرة - الجزيرة

١٩٧٧

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IN8781832



ERM036746098

مركز علاج نابلس



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



MDT form

A-Patient ID:

1- Name : غاده غنيم محمود

Sex:

3- Medical number:

Female

83034271

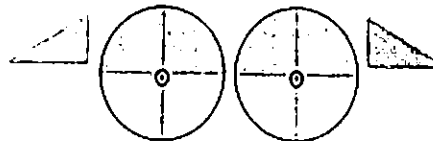
Old Med :

Old Med :

309-2018

2-ID number: 27709010108803

Patient Age : 48 Y



Family history

Family History

Phone

Phone

01127542609

BMI

BMI

REASON OF MDT

REASON OF MDT
RT/MT

marital status

Marital Status

NRS Score

NRS Score

GROUP

GROUP
IT

Date of MDT

Date Of MDT

2025-03-25

Co morbidity

Co Morbidity

Menopausal
status

Menopausal Status
PREMENOPAUSAL

Initial STAGE

Initial STAGE

Clinical data

Breast Imaging :

Mets Workup :

BX IDC BX COMMENT:	ER 8/8 ER COMMENT:	PR 8/8 PR COMMENT:	Her 2 Neu 3 Her 2 Neu COMMENT:	SISH SISH COMMENT:	KI67
--------------------------	--------------------------	--------------------------	--------------------------------------	-----------------------	------

T	N	M 1 (LUNG AND LIVER)
---	---	----------------------------

Sonomammography

PATIENT HISTORY

PREMENOPUSAL MBC TO LUNG AND LIVER

HISTORY-->2018 : CT3 N1 M0

neo adj. 4 FAC--> LT BCS 6/2018-->adj 2 taxotere stopped BECUSE OF AE-->PORT+ 17 CYCLES

HERCEPTIN LAST 12/2019

STARTED TAMOXIFEN 9/2018 TO 1/2022

1/2022 DYSNEA CT-->LUNG NODULES+ HFL

RECEIVED 18 WKS TAXOL+ HERCEPTIN-->REGRESSIVE COURSE

SRARTED AI+ZOLADEX+HERCEPTIN FROM 22/6 TILL 22/3/2023

3/2023-->METS HFL-->RECEIVED 9 WKS TAXOL+HERCEPTIN UPTO 22 CYCLE CTS-->PD OF HFL

started taxol+carbo+herceptin x 3 cycles--> 5/2023 PD xeloda+tykerb x 3 cycles --> RD

now in C9 xeloda,6 tykerb 29/11/2023 ct:pd of HFL

navelbine x 3 cycles-->PD .PS 1 -NORMAL LFT NO .VIS. CRISIS- HFLS = 6CM

3 cycle gemzar-->SD-->recieved kadcyla with very good response recived 12 cycles

CARBOPLATIN SINGLE AGENT

US GUIDED BIOPSY FROM HFL ER PR 618 HER 2 +3

NEW CT PD IN LIVER REC 1 CYCLE CARBOPLATIN

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

TO ADD HERCEPTIN NGO TO CARBOPLATIN FOR 6 CYCLES THEN HERCEPTIN + HORMONAL TTT

SIGNATURE

MEDICAL ONCOLOGY

MEDICAL ONCOLOGY

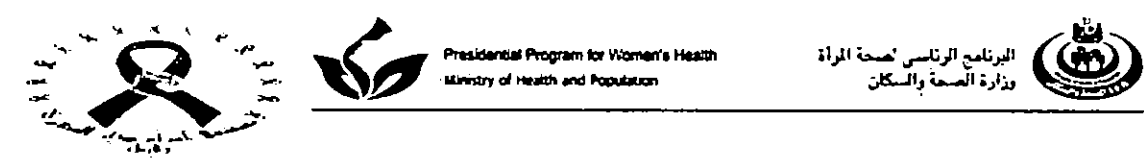
د/خالد عبدالعزیز

RADIOLOGY

RADIOLOGIST

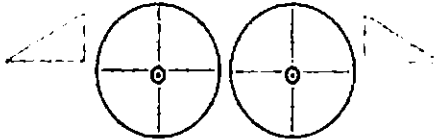
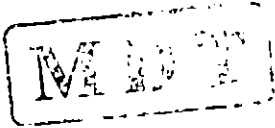
MDT Form

MDT Form (MDT Form)



MDT form

A-Patient ID: 1- Name : غادة غنيم مصمود Sex: Female 3- Medical number: 83034271 Old Med : 309-2018
2-ID number: 27709010108803 Patient Age : 47 Y



Family history	Family History	Phone	Phone	BMI	BMI	REASON OF MDT	REASON OF MDT
marital status	Marital Status	NRS Score	NRS Score	GROUP	GROUP	Date of MDT	Date of MDT
Co morbidity	Co Morbidity	Menopausal status	Menopausal Status	Initial STAGE	Initial STAGE	24/02/2024	

Clinical data

Breast Imaging :

Mets Workup :

BX IDC

ER 8/8

PR 8/8

Her 2 Neu 3

SISH

Ki67

BX COMMENT:

ER COMMENT:

PR COMMENT:

Her 2 Neu COMMENT:

SISH COMMENT :

T

N

M 1 (LUNG AND LIVER)

Sonomammography

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HERCEPTIN LAST 12/2019 STARTED TAMOXIFEN 9/2018 TO 1/2022
1/2022 DYSNEA CT-->LUNG NODULES+ HFL
RECEIVED 18 WKS TAXOL+ HERCEPTIN-->REGRESSIVE COURSE
SRARTED AI+ZOLADEX+HERCEPTIN FROM 22/6 TILL 22/3/2023
3/2023-->METS HFL-->RECEIVED 9 WKS TAXOL+HERCEPTIN UPTO 22 CYCLE CTS-->PD OF HFL
started taxol+carbo+herceptin x 3 cycles--> 5/2023 PD xeloda+tykerb x 3 cycles --> RD
now in C9 xeloda,6 tykerb 29/11/2023 ct:pd of HFL
navelbine x 3 cycles-->PD .PS 1 -NORMAL LFT NO .VIS. CRISIS- HFLS = 6CM

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

FOR 3 CYCLES GEMAZAR +CARBO

SIGNATURE			
MEDICAL ONCOLOGY	MEDICAL ONCOLOGY	RADIOLOGY	RADIOLOGY
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



MDT form

A-Patient ID:

1- Name : غاده غنيم محمود

Sex:

3- Medical number:

Female

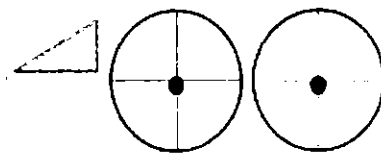
83034271

Old Med :

309-2018

2-ID number: 27709010108803

Patient Age : 46 Y



Family history

Phone

01148864478

BMI

REASON OF MDT

REMDT

marital status

NRS Score

01127542609

GROUP

M

Date of MDT

2023-09-19

Co morbidity

Menopausal
status

PREMENOPAUSAL

Clinical data

Breast Imaging :

Mets Workup :

BX:
IDC

ER:
8/8

PR:
8/8

Her 2 Neu :
3

SISH :

KI67 :

T

N

M

1 (LUNG AND
LIVER)

Sonomammography

PATIENT HISTORY

PREMENOPUSAL MBC TO LUNG AND LIVER ✓

HISTORY-->2018 : CT3 N1 M0

neo adj. 4 FAC---> LT BCS 6/2018--->adj 2 taxotere stopped BECUSE OF AE-->PORT+ 17 CYCLES

HERCEPTIN LAST 12/2019

STARTED TAMOXIFEN 9/2018 TO 1/2022 ✓

1/2022 DYSNEA

CT--->LUNG NODULES+ HFL

RECEIVED 18 WKS TAXOL+ HERCEPTIN-->REGRESSIVE COURSE

SRARTED AI+ZOLADEX+HERCEPTIN FROM 22/6 TILL 22/3/2023

3/2023-->METS HFL-->RECEIVED 9 WKS TAXOL+HERCEPTIN UPTO 22 CYCLE

CTS-->PD OF HFL

started taxol+carbo+herceptin x 3 cycles--> 5/2023 PD

xeloda+tykerb x 3 cycles --> RD

now in C5 xeloda, tykerb--> toxcity grade 2-3 vomiting

grade 2 diarrhea

MDT OUTCOME & PLAN

to cont same tkrb - xeloda with dose reduction

SIGNATURE

MDT Form
(DOCTYPE:MDT FORM)
دانشگاه



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الاستراتيجية
صحة
مصر

MDT FORM

/ / 20

G

Name	غادة عنب محمد	MRN	٢٠٩
Age	47+	ID	٤٠١٨
Menopausal status	premenop	Phone	٨٢٠٢٤٩٧١
NRS SCORE		Marital status	٠١١٤٨٨٦٤٤٧
BMI		Family history	-١١٢٧٥٩٤٦٠
CO morbidity		REASON OF MDT	

Clinical Data

BX:	IOC	R	L
ER: +	PR: +	HER 2: 3+	Ki67:
Sonomammography: premenop. MBC to Lung, Liver			
Mets Workup: NACT 4 FAC → LRBCs		6/2018 ady 2 Taxol Intercept 12/2019 Dyosin - CT	

Recommendations:

Mets to Lung & Liver.
18 wks Taxol + Intercept → PD Mark AS
+ Zoladex + Intercept till 3/223 Newly developed HPL
8+ Taxol + Intercept 9 wks Taxol + Carb 20 cycles Intercept
CT, 5/223 :- PD → Xeloda + Tykerb X 10 wks

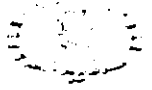
MDT OUTCOME & PLAN

PD → oral Naveltin X 3 wks → DP
3 cycles Gemzar → SD Rec Kadex
with very good Response Rec 12 cycles

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

after 12G - CT Non-Contrast progression
Course regarding sign & c... of previously
Hypoad... & c... full m... 2-1-18-



Patient Name	: الأستاذة / غاده غنيم محمود سالم	Request No.	: 13008160
Patient No.	: 27709010108803	Request Date	: 08/03/2025
Path No.	: 948/25	Verification Date	: 19/03/2025
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 47 Y	Department	: Out Patient
Referring Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Known case of breast cancer with hepatic focal lesion.

Nature of Specimen: Tru-cut biopsy, collected at 4/3/2025.

Gross Examination:

- Multiple grayish white tissue cores up to 1 cm, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:

- * Revealed cores of liver tissue showed positive metastatic deposits.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * IPT (Positive control showed appropriate reactivity):
- GATA3: Positive.
- ER: Moderate nuclear reaction in 60% of tumor cells (score 6 /8).
- PR: Strong nuclear reaction in 11% of tumor cells (score 6 /8).
- Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).
- Ki67: 30% nuclear reaction of tumor cells.

Diagnosis:

Liver tissue, Tru-cut biopsy:

Positive for metastatic deposits.

IPT:

- GATA3: Positive.
- ER: Positive (score 6 /8).
- PR: Positive (score 6 /8).
- Her2: Positive (score 3+).

Verified by :

DR.Menna Nabil

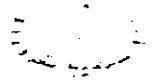
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Page 1 of 2

Printing By : Mrs.Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name : الأستاذة / غاده غنيم محمود سالم	Request No. : 13008160
Patient No. : 27709010108803	Request Date : 08/03/2025
Path No. : 948/25	Verification Date : 19/03/2025
Sex : Female	Financial Category : أورام دار السلام - نفقة الدولة
Age : 47 Y	Department : Out Patient
Referreing Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

- Ki67: Positive in 30% of tumor cells.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Prof. Dr. Lubna Omar.

Dr. Nermeen Mohammed.

Verified by :

DR.Menna Nabil



Patient Name : الأستاذة / غاده غنيم محمود سالم	Request No. : 138127
Patient No. : 27709010108803	Request Date : 11/01/2022
Path No. : 103/2022	Verification Date : 26/01/2022
Sex : Female	Financial Category : أورام دار السلام - نفقة الدولة
Age : 44 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Left breast mass.

Nature of the specimen: Tru-cut biopsy

Gross Examination:

Multiple greyish white and fatty tissue cores, upto 1 cm, totally submitted.

- Procedure: Tru-cut biopsy
- Localization: Left.
- Tumor extent
 - o Number: N/A
 - o Size: N/A
 - o Skin involvement: N/A
- Distance from margins: N/A
- Lymph nodes number and size: N/A
- Additional pathologic findings: N/A
- Immunohistochemistry technique: N/A
- Frozen section: N/A

Microscopic examination:

- Histological type: Totally fibrosed cores (scar tissue), free of malignancy.
- Grade (differentiation+ nuclear pleomorphism + mitosis): N/A
- Angioinvasion: N/A
- Ductal carcinoma in situ: N/A
- Tumor extent
 - o Number: N/A
 - o Size: N/A
 - o Skin involvement: N/A
- Lymph node involvement: N/A
- Resection Margins: N/A

Verified by :



Patient Name	: الأستاذ / غاده غنيم محمود سالم	Request No.	: 138127
Patient No.	: 27709010108803	Request Date	: 11/01/2022
Path No.	: 103/2022	Verification Date	: 26/01/2022
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 44 Y	Department	: Out Patient
Referring Doc :		Registered by	: .marwa mohamed youss

Pathology Report

- Histologic response to therapy: N/A
- Additional pathologic findings: N/A

Diagnosis:

Left breast mass, Tru-cut biopsy:

Totally fibrosed cores (scar tissue), free of malignancy.

Prof. Aml Hareedy
Dr. Mennatullah Atef

Verified by :

Printing Date : 26/01/2022 13:29:39

Page 2 of 2

Printing By : Ali mostafa

Pathology Report

Name : غاده غنيم محمود

Age: 40 years

Clinical data: Left breast cancer received neoadjuvant chemotherapy

Specimen Type: conservative Breast surgery

Date: 12/7/2018

File Number: 309 /2018

Gross Description:

Three specimens were received:

A/ sector of breast tissue 9x8 cm. serial sections revealed firm grey white mass 2.8x1.5 cm. The mass located (1 cm, 2 cm, 2.5 cm and 0.5 cm) away from (superior, lateral, deep and medial) margins respectively .

B/ Fibrofatty tissue piece 5x4cm.

C/ Fibrofatty tissue piece 4x4cm entangling 7 lymph nodes up to 1x1cm.

Microscopic Examination:

A) Sections examined revealed breast tissue infiltrated by malignant tumour cells arranged in acini, trabeculae and solid nests surrounded by desmoplastic stroma. The cells showed moderate anaplasia. The tumour showed partial therapeutic response (Tumour necrosis index 20%) with minimal in situ component (<5%), no lymphovascular invasion nor perineural invasion seen. All rejection margins were free of tumour tissue.

B) Sections examined from extending margin showed fibrocystic changes and fibroadenomatoid hyperplasia with no tumour tissue seen.

CT3N1

Pathology Report

NAME: غاده غنيم محمود

Age: 50 years.

Clinical data: left breast mass.

Specimen Type: True cut biopsy.

Date : 19/3/2018

Gross Description:

Three greyish white tissue cores 1cm, 0.5cm. and 0.5 long. Totally submitted.

Microscopic Examination:

Microscopic examination of sections prepared revealed Invasive Duct Carcinoma. G2. Surrounded by moderate desmoplastic reaction. No intraductal component.

Occassional lympho-vascular emboli were detected.

Diagnosis:

Breast mass , true cut biopsy, invasive duct carcinoma. G2

Prof Dr: Essam Ayad.

Dr: Marwa Abd El kader

خطابات أورام

Patient Name : غاده غنيم محمود

sex : female Age: 50

Hosp.Word : TC

Patient status : outpatient

Path.No. : 0000

IHC. No : 10434

Referring Doctor: اورام السلام

Date : 23-05-2018

Provisional Diagnosis: Request for ER/PR/cerbB2

Immunohistochemistry Report

IMMUNOHISTOCHEMISTRY

Immunostaining effectuated on 5 µm thick paraffin sections, the heat induced epitope retrieval using microwave, the ChemMate DAKO Envision, DAKO autostainer and the following antibodies:

- Estrogen receptors: moderate nuclear staining of 50% of neoplastic cells.
- Progesterone receptors: moderate nuclear staining of 10% of neoplastic cells.
- CerbB2: strong complete membranous staining of all the neoplastic cells.

CONCLUSION

Invasive mammary carcinoma, core biopsy:

- Estrogen and Progesterone receptors positive.
- CerbB2 positive.

NB

core biopsy, score is not applicable

Enclosed :

Slides :

Blocs :

مدير معمل الصبغات الهستومناعية

ا. د / هويدا عبدالعال

Director Immunohistochemical Lab

Prof. Howayda Abd El Ali

30-05-2018

Name: ghada ghomeem mahmoud

Date: 11 June 2020

Soft tissue ultrasound examination of both breasts revealed: -

Follow up after left breast lumpectomy revealed:

Left breast:

- > Skin thickening seen at the upper outer quadrant and areolar region
- > The breast parenchymal shows multiple hypoechoic line of interstitial edema
- > There is irregular hypoechoic area under the surgical scar measuring 11x12x16 mm, with no calcification (? Surgical scar).
- > No other mass or cystic changes.
- > One enlarged axillary LN seen, measuring 21x9 mm, it has fatty hilum and thin cortex..

Right breast:

- > Predominantly fibro-glandular breast parenchyma with no areas of architectural distortion.
- > No masses or cysts could be detected.
- > Normal duct system.
- > Normal nipple areola complexes.
- > Intact skin and subcutaneous tissues.
- > No enlarged right axillary LNs..

OPINION:

Edematous changes seen at the left breast with ? surgical scar for close follow up BIRADS III.

Best regards

Dr / Daaa Ahmed MS-FEBR

The Breast Imaging and Reporting Data System for classifying the radiological diagnosis of breast lesions:

- BIRADS 0: needs further investigations (0%).
- BIRADS 1: normal findings.
- BIRADS 2: definitely benign.
- BIRADS 3: probably benign (2 %).
- BIRADS 4: probably malignant (50 %).
- BIRADS 5: malignant (98-100 %).
- BIRADS 6: proven pathologically malignant.

Name :

Gada gonlem Mahmoud

Date: 16-Mar-2020

ULTRASOUND BOTH BREASTS

• Clinical data : 42 y female with history of left breast cancer surgical excision followed by chemotherapy and radiotherapy

– RIGHT breast

– No solid or cystic lesion could be detected

• No pathological macro ($>0.5\text{mm}$) or micro-calcifications could be seen by US.

– No Evidence of pathologically enlarged intramammary or axillary lymph nodes

– LEFT breast

• *an irregular hypoechoic area measuring $19 \times 8\text{mm}$ has (angulated speculated) margins & posterior shadowing with no internal vascularity seen at 12 o'clock (the site of the previous surgery) with slightly architectural distortion*

• No pathological macro ($>0.5\text{mm}$) or micro-calcifications could be seen by US.

• No Evidence of pathologically enlarged intramammary or axillary lymph nodes.

CONCLUSION:

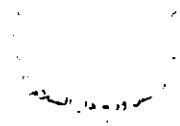
○ Left breast lesion consistent with ?surgical scar for further evaluations by mammography

○ Right breast normal US (BIRADS 1)

Much obliged, yours

NERMEEN RAMADAN.MD

0	Need additional imaging evaluation
1	Normal breast
2	Benign findings
3	Probably benign for short interval follow up.
4	Suspicious for biopsy (A mild B moderate C high)
5	Highly suggestive malignancy
6	Pathologically proven malignant



Patient Name: Ghada Ghonaim Mahmoud Salem	Date : 27/10/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 42 Year(s)
Medical ID: 309/2018	Pat. ID: 13109

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

Left breast conserving therapy

Technique

Bilateral MLO and CC views

Bilateral breast ultrasound

Report:

Scattered fibro glandular density (ACR B)

Left breast:

Operative bed parenchymal distortion and scarring with no sizable mass

No suspicious mass lesions, micro calcifications

No sizable collection.

No regional lymphadenopathy

Right breast:

- No suspicious micro calcifications or architectural distortion detected.
- No spiculated mass
- Normal retro-areola ducts
- Normal skin thickness, and nipple-areola complex
- Nonspecific axillary LN.

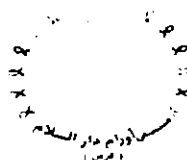
Opinion:

Post left BCS status showing no evidence of recurrence (BIRADS 2)

Normal study of the right breast (BIRADS 1)

Signature Dr. Wafaa Ali

27/10/2024 1:29 PM



Patient Name: Ghada Ghonaim Mahmoud Salem	Date : 02/10/2023
Exam : MAMMOGRAPHY BOTH	Age : 41 Year(s)
Medical ID: 309/2018	Pat. ID: 13109

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

- Left breast conserving therapy

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

Scattered fibro glandular density (ACR B)

Left breast:

- Operative bed parenchymal distortion and scarring with no sizable mass
- No suspicious mass lesions, micro calcifications
- No sizable collection.
- No regional lymphadenopathy

Right breast:

- No suspicious micro calcifications or architectural distortion detected.
- No spiculated mass
- Normal retro-areola ducts
- Normal skin thickness, and nipple-areola complex
- Nonspecific axillary LN.

Opinion:

- Post left BCS status showing no evidence of recurrence (BIRADS 2)
- Normal study of the right breast (BIRADS 1)

Signature Dr. Peter Nashat

02/10/2023 11:43 AM

Patient Name: Ghada Ghonaim Mahmoud	Date : 10/10/2022
Exam : MAMMOGRAPHY BOTH'	Age : 40 Year(s)
Medical ID: 309/2018	Pat. ID: 13109

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

Left breast conserving therapy

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)

Left breast:

- Operative bed parenchymal distortion and scarring with no sizable mass
- No suspicious mass lesions, micro calcifications
- No sizable collection.
- No regional lymphadenopathy

Right breast:

- No suspicious micro calcifications or architectural distortion detected.
- No spiculated mass
- Normal retro-areola ducts
- Normal skin thickness, and nipple-areola complex
- Nonspecific axillary LN.

Opinion:

- Post left BCS status showing no evidence of recurrence (BIRADS 2)
- Normal study of the right breast (BIRADS 1)

Best regards,

Dr. Alamin Haider

Dr. Amira H Radwan MD,
Breast Imaging consultant,
NCI, Cairo University

مركز نورام دار السلام
جسر مصر سابقا

Patient name: غادة غنيم محمود

Date: 1.09.2021

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

Patient had left CBS in 5/2018

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

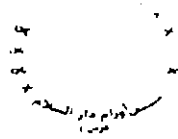
Report:

- The breasts are scattered fibroglandular parenchyma (ACR b).
- No right breast suspicious mass lesions, micro calcifications or architectural distortion detected.
- Left breast UOQ focal area of parenchymal distortion owing to prior surgical interference, with post-operative scarring and mild edema pattern in the form of mild skin thickness.
- Complementary ultrasound also showed left breast UOQ focal parenchyma distortion, owing to prior surgical interference with clear operative bed with no evidence of recurrent mass lesions.
- Right breast prominent parenchyma with mild nodulation and impressive of adenosis.
- Nonspecific right axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free left infra and supra clavicular regions.

Opinion:

- left breast focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative sequel, with free operative bed with no evidence of recurrent mass lesions (BIRADS 2)
- right breast fibroadenosis (BIRADS 2)

Best regards,
Dr/ Nehal hamdey, FEBR, MSC.



Patient Name: Ghada Ghonaim Mahmoud Salem	Date : 15/05/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 43 Year(s)
Medical ID: 309/2018	Pat. ID: 13109

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

❖ Compared to the previous contrast study dated 22-02-2025.

1. Chest:

- Status post Left BCS showing:
- **Stationary course regarding the:**
 - Operative bed scar related/ inseparable from underlying left pectoralis muscle.
 - Left lung upper lobe anterior subpleural fine reticular bands with right middle atelectatic band, likely post radiotherapeutic sequel with still seen in lingular segment.
 - Reticulo-nodular density detected all through the lung segments.
 - Bilateral basal atelectasis.
- No hilar or mediastinal sizable pathologically enlarged lymph nodes.
- No evidence of sizable suspicious pulmonary nodules.
- No evidence of pleural effusion, thickening or masses seen.
- Normal cardiac size. No pericardial effusion detected.
- Normal appearance of thoracic aorta and great vessels.

2. Abdomen and pelvis:

- The liver is still seen enlarged showing **regressive course** regarding the size and extension of previously noted hypodense discrete and confluent focal lesions, the largest is still seen at segment VIII currently measuring about 3x3.6 cm comparing to 6.8x4.5 cm.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.



Patient Name: Ghada Ghonaim Mahmoud Salem	Date : 15/05/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 43 Year(s)
Medical ID: 309/2018	Pat. ID: 13109

- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Stationary subcentimetric para-aortic lymph nodes, the largest is seen at the left side and measures about 10x8 mm.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Still depicted homogenously enhancing uterine rounded lesion in fundus for ultrasound correlation.
- Normal appearance of the rest pelvic organs.
- No abdominal collections.

3. **Skeletal survey:**

- No definite worrisome lytic or sclerotic bone lesions.
- Still noted of few pelvic sclerotic bone lesions, likely bone islands.
- Still noted L5 First degree lytic spondylolisthesis with bilateral L5 pars interarticularis break.

OPINION:

- **Regressive course regarding the hepatic focal lesions.**
- **Rather stationary course** regarding the rest of the study.

Dr. Shaimaa Khaled, MSc.

Dr. Reham Anwar, FEBR, MD



Patient Name: Ghada Ghonaim Mahmoud Salem	Date : 27/11/2024
Exam : C.T CHEST WITH CONTRAST	Age : 42 Year(s)
Medical ID: 309/2018	Pat. ID: 13109

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

❖ Compared to the previous study dated 12-9-2024.

1. Chest:

- Status post Left BCS showing:
- **Stationary course regarding the:**
 - Operative bed scar related/ inseparable from underlying left pectoralis muscle.
 - Left lung upper lobe anterior subpleural fine reticular bands with right middle atelectatic band, likely post radiotherapeutic sequel with still seen in lingular segment.
 - Reticulo-nodular density detected all through the lung segments.
 - Subtle progressive course of bilateral basal atelectasis in current study.
- No hilar or mediastinal sizable lymph nodes.
- No evidence of pulmonary nodules.
- No evidence of pleural effusion, thickening or masses seen.
- Normal cardiac size. No pericardial effusion detected.
- Normal non-enhanced appearance of thoracic aorta and great vessels.

2. Abdomen and pelvis:

- The liver is still seen enlarged showing **subtle regression Course** regarding the size and number of previously noted hypodense discrete and confluent focal lesions, the largest is still seen at segment VIII measuring about 2.1x1.8cm comparing to previously measures about 2.5 x 2.2 cm.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.



Patient Name: Ghada Ghonaim Mahmoud Salem	Date : 27/11/2024
Exam : C.T CHEST WITH CONTRAST	Age : 42 Year(s)
Medical ID: 309/2018	Pat. ID: 13109

- No sizable pelvi-abdominal lymph nodes.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Still depicted of rounded mass with currently homogeneous enhancing in fundus likely fundus for pelvic US correlation.
- Normal appearance of the rest pelvic organs.
- No abdominal collections.

3. **Skeletal survey:**

- No definite worrisome lytic or sclerotic bone lesions.
- Still noted of few pelvic sclerotic bone lesions, likely bone islands.
- Still noted L5 First degree lytic spondylolisthesis with bilateral L5 pars interarticularis break.

OPINION:

- **Rather stationary course** regarding chest, abdomen and pelvis.

Dr.Maitha Fadhl

Dr. Reham Anwar, FEBR, MD



Patient Name: Ghada Ghonaim Mahmoud Salem	Date : 06/03/2023
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 40 Year(s)
Medical ID:	13109

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

A known case of breast cancer.

Compared to the previous study dated 18/12/2022 the current study shows:

1. Chest:

- Status post left BCS with still noted ill-defined density close to the pectoralis muscle likely scar.
- Still noted right upper lobe posterior segment reticulo-nodular density.
- Still noted left lung upper lobe anterior subpleural fine reticular bands with right middle atelectatic band. Right lower lung subpleural mosaic pattern.
- Stable right lower lobe atelectasis.
- No hilar or mediastinal sizable lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size. No pericardial effusion detected.
- Normal non-enhanced appearance

2. Abdomen and pelvis:

- The liver is enlarged with newly appreciated segment VI hypo enhancing focal lesion, it measures 1.5 cm. No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.



Patient Name: Ghada Ghonaim Mahmoud Salem	Date : 20/05/2023
Exam : C.T CHEST WITH CONTRAST	Age : 41 Year(s)
Medical ID:	13109

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

**** A known case of breast cancer.**

**** As compared to the previous study dated 06/03/2023;**

1. Chest:

Status post left BCS intervention,

- **Stable appearance of the operative bed** ill-defined scar close to the pectoralis muscle.
- **Still noted** right upper lobe posterior segment reticulo-nodular density.
- **Still noted** left lung upper lobe anterior subpleural fine reticular bands with right middle atelectatic band. Right lower lung subpleural mosaic pattern.
- **Still noted** right lower lobe atelectasis.
- No hilar or mediastinal sizable lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size. No pericardial effusion detected.
- Normal non-enhanced appearance

2. Abdomen and pelvis:

- The liver is enlarged with **progressive course** regarding the size of the two hypo-enhancing focal lesions, the largest is currently appreciated subcapsular at segment V subcapsular measuring about 33x28 mm as compared to ill defined 18 mm, and segment VI currently measuring about 21x11 mm compared to 15x11 mm. No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.

- Normal size & parenchymal texture of the spleen with no focal lesions seen.



Patient Name: Ghada Ghonaim Mahmoud Salem	Date : 27/09/2023
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 41 Year(s)
Medical ID: 309/2018	13109

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

**** A known case of breast cancer.**

**** As compared to the previous study dated 26/7/2023,**

1. Chest:

Status post left BCS intervention,

- **Stable appearance of the operative bed** ill-defined scar close to the pectoralis muscle.
- **Still noted** right upper lobe posterior segment reticulo-nodular density.
- **Still noted** left lung upper lobe anterior subpleural fine reticular bands with right middle atelectatic band. Right lower lung subpleural mosaic pattern.
- **Still noted** right lower lobe atelectasis.
- No hilar or mediastinal sizable lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size. No pericardial effusion detected.
- Normal non-enhanced appearance

2. Abdomen and pelvis:

- The liver is enlarged with **regressive course** regarding the size of the two hypo-enhancing focal lesions, the largest is currently appreciated subcapsular at segment V subcapsular measuring about 20 X 13 mm compared to 33x28 mm with overlying capsular retraction. No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.

Patient Name: Ghada Ghonaim Mahmoud	Date : 18/12/2022
Exam : C.T CHEST WITH CONTRAST	Age : 40 Year(s)
Medical ID:	

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

A known case of breast cancer.

Compared to the previous study dated 03/09/2022 the current study shows:

1. Chest:

- Status post left BCS with still noted ill-defined density close to the pectoralis muscle likely scar.
- Still noted right upper lobe posterior segment reticulo-nodular density.
- Still noted left lung upper lobe anterior subpleural fine reticular bands with right middle atelectatic band. Right lower lung subpleural mosaic pattern.
- Stable right lower lobe atelectasis.
- No hilar or mediastinal sizable lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size. No pericardial effusion detected.
- Normal non-enhanced appearance

2. Abdomen and pelvis:

- The liver is enlarged with no currently noted focal lesions. No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- No lytic or sclerotic bone lesions; apart from few pelvic sclerotic bone lesions, likely bone islands.



Patient Name: Ghada Ghonaim Mahmoud	Date : 03/09/2022
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 40 Year(s)
Medical ID:	Pat. ID: 13109

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

A known case of breast cancer.

Compared to the previous study dated 12-6-2022 the current study shows:

1. Chest:

- Status post left BCS with still noted ill-defined density close to the pectoralis muscle.
- Still noted right upper lobe posterior segment reticulo-nodular density.
- Still noted left lung upper lobe anterior subpleural fine reticular bands with right middle atelectatic band. Right lower lung subpleural mosaic pattern.
- Regression of the right lower lobe atelectasis.
- No hilar or mediastinal sizable lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size. No pericardial effusion detected.
- Normal non-enhanced appearance

2. Abdomen and pelvis:

- The liver is enlarged with no currently noted focal lesions. No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.



Name : Ghada Ghoenim

Date : 4-4-2022

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

A known case of breast cancer.

Compared to the previous study dated 4-1-2022 the current study shows:

1. Chest:

- Status post left BCS with LOQ operative bed shows deeply seated soft tissue heterogeneously enhanced lesion inseparable from the underlying pectoralis muscle still measuring about 23x22 mm.
- ***Regressive course*** regarding the previously noted right lung upper lung lobe posterior segmental nodule measuring about 1.9x1.2cm compared to 3x2.5 cm in the previous study.
- Still noted left lung upper lobe anterior subpleural fine reticular bands with right middle atelectatic band. Right lower lung subpleural mosaic pattern.
- No hilar or mediastinal sizable lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size. No pericardial effusion detected.
- Normal non-enhanced appearance

2. Abdomen and pelvis:

- The liver is enlarged showing ***regressive course*** regarding the size of the previously noted hypodense focal lesions, with largest seen at the right lobe segment VI measuring about 1x1cm compared to 2.3x2cm in the previous study. The lesions show homogenous contrast enhancement with some showing marginal enhancement in the porto-venous phases with wash out in the delayed phases
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries. No intra-hepatic or extra-hepatic biliary radicles dilatation.



Name : Ghada Ghoenim

Date : 04-01-2022

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

A known case of breast cancer.

1. Chest:

- Status post left BCS with LOQ operative bed shows deeply seated soft tissue heterogeneously enhanced lesion inseparable from the underlying pectoralis muscle measuring about 23x22 mm.
- Right lung upper lung lobe posterior segment shows irregular mass measuring about 30x25 mm.
- Left lung upper lobe anterior subpleural fine reticular bands with right middle atelectatic band.
- Right lower lung subpleural mosaic pattern.
- No hilar or mediastinal sizable lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size. No pericardial effusion detected.
- Normal non-enhanced appearance

2. Abdomen and pelvis:

- The liver is enlarged showing few hypodense focal lesions, some show homogenous contrast enhancement and others show marginal enhancement in the portovenous phases with wash out in the delayed phases, the largest is seen at the right lobe segment VI measures about 23x20 mm.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.



Patient Name: Ghada Ghonaim Mahmoud Salem	Date : 22/02/2025
Exam : C.T CHEST	Age : 42 Year(s)
Medical ID: 309/2018	Pat. ID: 13109

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

❖ Compared to the previous contrast study dated 27-11-2024.

1. Chest:

- Status post Left BCS showing:
- **Stationary course regarding the:**
 - Operative bed scar related/ inseparable from underlying left pectoralis muscle.
 - Left lung upper lobe anterior subpleural fine reticular bands with right middle atelectatic band, likely post radiotherapeutic sequel with still seen in lingular segment.
 - Reticulo-nodular density detected all through the lung segments.
 - Bilateral basal atelectasis in current study.
- No hilar or mediastinal sizable lymph nodes.
- No evidence of pulmonary nodules.
- No evidence of pleural effusion, thickening or masses seen.
- Normal cardiac size. No pericardial effusion detected.
- Normal non-enhanced appearance of thoracic aorta and great vessels.

2. Abdomen and pelvis:

- The liver is still seen enlarged showing **progressive course** regarding the size and extension of previously noted hypodense discrete and confluent focal lesions, the largest is still seen at segment VIII currently measuring about 6.8x4.5 cm comparing to 2.1x1.8cm on non-contrast basis.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen on non-contrast basis.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.



Patient Name: Ghada Ghonaim Mahmoud Salem	Date : 22/02/2025
Exam : C.T CHEST	Age : 42 Year(s)
Medical ID: 309/2018	Pat. ID: 13109

- Stationary subcentimetric para-aortic lymph nodes, the largest is seen at the left side and measures about 10x8 mm.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- The previously noted rounded mass in fundus couldn't be depicted in today's non-contrast study.
- Normal appearance of the rest pelvic organs.
- No abdominal collections.

3. **Skeletal survey:**

- No definite worrisome lytic or sclerotic bone lesions.
- Still noted of few pelvic sclerotic bone lesions, likely bone islands.
- Still noted L5 First degree lytic spondylolisthesis with bilateral L5 pars interarticularis break.

OPINION:

- **Progressive course regarding the hepatic focal lesions.**
- **Rather stationary course** regarding the rest of the study.

Dr/ Maie Khattab, MSc.

Dr. Reham Anwar, FEBR, MD



Total DLP (mGy.cm):

Patient Name: Ghada Ghonaim Mahmoud Salem	Date : 20/04/2024
Exam : C.T ABDOMEN & PELVIS	Age : 42 Year(s)
Medical ID: 309/2018	Pat. ID: 13109

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

- **Known case of breast cancer.**
- **Compared to the previous study dated 6-2-2024.**

1. Chest:

- Status post left BCS intervention showing stationary course regarding the:
 - Operative bed ill-defined scar close to the pectoralis muscle.
 - Left lung upper lobe anterior subpleural fine reticular bands with right middle atelectatic band, likely post radiotherapeutic sequel.
 - Right lower lobe atelectasis.
- Reduced previous Right upper lobe posterior segment reticulo-nodular density.
- No hilar or mediastinal sizable lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size. No pericardial effusion detected.
- Normal non-enhanced appearance

2. Abdomen and pelvis:

- The liver is enlarged showing based on today's non-contrast study rather stationary course regarding the previously noted hypodense discrete and confluent focal lesions, the largest is currently still appreciated at segment V measuring about 50 X 38 mm.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.



Total DLP (mGy.cm):

Patient Name: Ghada Ghonaim Mahmoud Salem	Date : 20/04/2024
Exam : C.T ABDOMEN & PELVIS	Age : 42 Year(s)
Medical ID: 309/2018	Pat. ID: 13109

- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

3. **Skeletal survey:**

- No definite worrisome lytic or sclerotic bone lesions.
- Still noted few pelvic sclerotic bone lesions, likely bone islands.
- Still noted L5 First degree lytic spondylolisthesis.

OPINION:

- **Overall stationary course of the study on non-contrast basis.**

Dr. Ali Elsisy, MSc.

Dr. Reham Anwar, FEBR, MD